

# AI-native sourcing and bridging of de-risked (approved) Japanese pharmaceutical drugs

EF - May 2026

Between 2005 and 2022, **78 drugs were approved in Japan but never in the United States** (JAMA Network Open, 2025). Each one is a de-risked molecule with multi-year human data, sitting unbridged because the originator is regionally focused. The canonical comp is **Crystallys Therapeutics**: Catalys Pacific in-licensed Fuji Yakuhin's URAT1 inhibitor dotinurad (approved in Japan since 2020, 1.2M patients treated), spun a NewCo, and raised a **\$205M Series A** in September 2025 to run two global Phase 3 trials. We are disrupting the relationship-heavy nature of Pharma BD. After finding an asset, we create a NewCo, led by experienced disease-area operators, accelerated by an AI-native sourcing and regulatory layer that compresses the document-heavy parts of the bridging package. **We are building the tools that allow us to analyze the universe of Japanese opportunities and execute at a higher throughput and faster per asset.**

---

## Why now

The patent cliff is forcing a major in-licensing scramble — roughly \$300B of branded pharma revenue loses exclusivity between 2025 and 2030, with five of the top ten pharma companies facing over 50% revenue exposure (Evaluate, Jan 2026). That capital has run east. Chinese biotech out-licensing hit a record ~\$137B in 2025, DealForma reports China now dominates the big licensing deals of Q1 2026, and average upfronts on Western-China deals have risen 230% from \$52M in 2022 to \$172M in early 2026 (DealForma / Evaluate, Feb 2026). But the China trade now carries real friction: the BIOSECURE Act was signed into law on December 18, 2025 as Section 851 of the FY2026 NDAA, NMPA data-integrity audits are a recognised diligence risk, and prices have re-rated to the point analysts publicly call out that "it's not a bargain basement anymore." Independently, the FDA has grown more receptive to well-constructed foreign data, including Japanese post-marketing safety databases and real-world evidence. Since the 2022 (sintilimab) rejection, the agency has also required multi-regional, ICH E17-compliant pivotal trials — a standard Japanese and Korean trial designs are generally built to meet. The live proof point is Crystallys Therapeutics, which licensed dotinurad from Japan's Fuji Yakuhin and persuaded the FDA to skip Phase 2 entirely and go straight to global Phase 3 on the back of the Japanese approval and >2.2M-patient post-marketing safety database — saving, by the CEO's own account, ~5 years and \$30–40M.

---

## Product

**Sourcing.** Continuous ingestion of PMDA approval reports, jRCT trial entries, MHLW disclosures, Japanese pharma annual reports and licensing press releases. Cross-referenced against FDA Drugs@FDA and EMA EPAR to identify drugs approved in Japan whose ex-Asia rights remain unallocated. Inputs: PMDA, jRCT, MHLW, EDINET, Drugs@FDA, EPAR. The product is a queryable asset database — a record of every drug approved in Japan, with current allocation status (rights held, rights free, rights ambiguous) and a first-pass bridgeability score.

**Bridge analysis.** Automated parsing of the public PMDA review report. From it: ICH E5 ethnic-sensitivity profile, comparator gaps versus Western standard of care, formulation complexity, IP runway via J-PlatPat and Espacenet. Output: a go/no-go memo with indicative cost, timeline and bridging-study requirement. Bridging an asset is a multi-year sequence of expert workflows. The early ones (ICH E5 assessment, comparator analysis, IP runway) are document- and precedent-heavy and we automate them first. The later ones (Phase 3 protocol design, CMC, NDA assembly) need humans on the first pass, but each pass creates the structured pattern that can run as an agent on the next asset. The asset is downstream value; the agent library is the compounding moat.

**Develop.** Spin a NewCo, license ex-Asia rights into it, and recruit a disease-area-specialist CEO with heavy equity. Candidates come from an operator directory built per therapeutic area — credible CEOs and CMOs scraped from FDA approval credits, ClinicalTrials.gov PI records, prior NewCo leadership and SEC S-1 filings. The NewCo files the IND, runs Phase 3 in the US/EU, files the NDA and commercialises directly or through a specialty pharma partner. Crystallys is the precedent: roughly \$200M raised to take one asset of this kind through to NDA. James Mackay had taken Ardea's lesinurad to FDA approval — the first URAT1 inhibitor — when Catalys built Crystallys around him for dotinurad.

---

## Incumbents

**Catalys Pacific.** Tokyo/SF VC, founded 2019. Decade of relationships with Japanese mid-cap pharma; Eisai, Takeda and Chugai are Strategic Partners. They run the exact playbook we want to run: Crystallys (dotinurad, approved Japan 2020 → US Phase 3), Pathalys (upacalcet, approved Japan 2021 → \$105M Series B for US Phase 3), Phathom (vonoprazan, Takeda's Takecab → FDA-approved as VOQUEZNA).

**AN Venture Partners.** Tokyo/SF VC, \$200M Fund I closed July 2025, allied with ARCH. Adjacent rather than direct: invests in early-stage Japanese science (Capacity Bio, Typewriter, City Therapeutics, Imbria), not approved-elsewhere assets. They bridge Japanese science to US capital; we bridge Japanese-approved drugs to Western development.

**Mid-cap pharma BD.** Most pharma BD targets pre-clinical and early-stage assets; in-licensing already-approved drugs is rare. When it happens it is deal-by-deal: Japanese originators (Bioprojet, Eisai) out-license directly, or Western specialty pharma form one-off NewCos around a single asset (Phathom for Takeda's vonoprazan).

---

## Founders

**Zenyi, Co-founder.** Shipped American Express's first production AI agent, leading a 12-engineer team. Forward-deployed AI engineer at the NHS. Defence AI research at the UK's institute for AI and data science. Turing Fellow.

**Hugo, Co-founder.** AI-first biotech investing at Averin Capital. Healthcare and consumer PE at PAI Partners (\$33bn AUM). Goldman Sachs (Arm IPO). Co-founded Netssio; Blackstone Future Founders Fellow.

*Sources: PMDA approval database; JAMA Network Open 2025 (Kasahara et al., approval disparities US/Japan); ICH E5(R1) guideline; DealForma 2025; Crystallys Therapeutics Series A press release, September 2025.*